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Professionalism Cases

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OD3

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Objectives

- Recognize common professional communication issues for osteopathic medical students, residents, and physicians
- Describe how professional communication may impact patient care
- Discuss professional team interaction, self-directed learning, and self-attestation
- Analyze professional interactions for best practices in response to common situations

Interpersonal and Communication Skills 2: Interprofessional and Team Communication

Level 1	Level 2	Level 3	Level 4	Level 5
<u>Respectfully</u> requests a routine consultation Uses language that <u>values all members of</u> the health care team <u>Provides</u> prompt, objective and honest feedback on evaluations	Clearly and concisely explains clinical scenario and rationale for consultation Communicates information effectively with all health care team members <u>Solicits</u> feedback on performance as a member of the health care team	Checks own understanding of consultant recommendations Uses active listening to adapt communication style to fit team needs Communicates concerns and provides feedback to peers and learners	Coordinates recommendations from different members of the health care team to optimize patient care Communicates feedback and constructive criticism to superiors	Role models flexible communication strategies that value input from all health care team members, resolving conflict when needed Facilitates regular health care team-based feedback in complex situations
☐	☐	☐	☐	☐
Comments: Not Yet Completed Level 1 ☐				

Professionalism 2: Accountability and Conscientiousness

Level 1	Level 2	Level 3	Level 4	Level 5
Completes tasks and assigned responsibilities, with guidance	Independently completes tasks and assigned responsibilities in a timely manner with appropriate attention to detail in routine situations	Independently completes tasks and assigned responsibilities in a timely manner with appropriate attention to detail in complex or stressful situations	Proactively communicates with program staff members regarding situations that may impact own ability to complete tasks and responsibilities in a timely manner	Intervenes in situations that impact others' ability to complete tasks and responsibilities in a timely manner
Arrives on time and prepared for work	Takes responsibility for personal actions and performance	Admits errors and proposes remediation as necessary		
☐	☐	☐	☐	☐

Comments:

Not Yet Completed Level 1

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Randomized Controlled Trial

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Effect of Increased Interprofessional Familiarity on Team Performance, Communication, and Psychological Safety on Inpatient Medical Teams: A Randomized Clinical Trial

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Conclusions and relevance: In this randomized clinical trial, increased familiarity between nurses and residents promoted more rapid improvement of nursing perception of team relationships and, over time, led to higher team performance on complex cognitive tasks in medical simulations. Medical centers should consider team familiarity as a potential metric to improve physician-nursing teamwork and patient care.

Professional interactions improve patient care

- At the 12-month simulation, intervention teams:
 - work as 1 unit (100% vs 62%; $P = .003$)
 - **negotiate with the patient** (61% vs 10%; $P = .001$)
 - support other team members (61% vs 24%; $P = .02$)
 - communicate as a team (56% vs 19%; $P = .02$)
 - more successful in **achieving the correct simulation case outcome** of negotiating a specific insulin dose with the patient (67% vs 14%; $P = .001$)
 - input of all clinical practitioners was valued (95% vs 53%; $P < .001$)
 - feedback between practitioners was delivered in a way to promote positive interactions (90% vs 60%; $P = .003$).

Patient – Student – Faculty Interactions

Case 1 - Odd numbered groups

- You are in Student Run Free Clinic and are starting to treat your patient with OMT. The patient asks "Have you done this before? Are you good at it?"

Case 2 - Even numbered groups

- At SRFC, you present your patient to the preceptor, you then both go see the patient. The preceptor finds different structural findings and starts to explain and demonstrate the differences.

Patient – Student – Faculty Interactions

Case 1 - Odd numbered groups

- Main Issue? reassuring patient
- How do you respond to the patient? been doing for awhile and have signed off on these skills with a license professional practiced with them frequently but if not there is attending with me
- Should you continue to care for this patient? yes unless pt disagrees
if you feel discomfort I'll stop but should not cause harm - ways to go
- How might your choice of response affect patient care?

being stressed during therapy will have negative impact on them

how you hold yourself is important - calm comforting present while being aware skill level to build trust and allowing you to practice the skill and builds trust with preceptor to allow you to practice more

Case 2 - Even numbered groups

- Main issue? student and preceptor disagreeing in front of pt
student: I found something else
agree with preceptor bc they have more experience
- How the student respond?
- What if you still disagree? can ask f/u after leaving the room
- How will this affect patient care?

just laying hands on can change things
read between the lines - people will show you things in the moment (unless they're being rude)

depends on how the preceptor acts too
having an argument in front of patient loses patient

Interactions with Medical Team

Case 1 - Odd numbered groups

- You observe the MA taking vital signs and then proceed to tell the patient that the patient's BP of 118/76 was high. As a med student, you rush over and say, "no, stop! The blood pressure is not high!" in front of patient. The MA hunches her shoulders and looks at the floor while she walks away.

Case 2 - Even numbered groups

- Prior to giving the patient a vaccine, the MA usually gives the patient an informational sheet to read. You see the MA walk out of the patient's room, so you go in and ask the patient if it's ok to give them the vaccine. The patient states "yes". Right after you administer the vaccine, the MA steps into the room with the vaccine informational sheet. You realize that the MA hadn't given the vaccine information to the patient yet.

Interactions with Medical Team

Case 1 - Odd numbered groups

- What is the main issue? cx between student and MA
MA made mistake and speaking out of line in front of patient
- What do you think of the medical student's approach to the situation? maybe not if first time working with them, and shouldn't be done in front of the patient
- What do you think of the MA's reaction? sensitive and shouldn't be interpreting
- How might this interaction affect patient care? patient will be stressed and will not trust team
- What would you do differently?
not argue in front of patient - if patient needs reassurance ask attending
too dramatic of a response - talk to MA in private

student retake blood pressure

Case 2 - Even numbered groups

- What is the main issue?
- How do you feel about what happened?
- How would you approach talking to the MA about this?
- What would you say to the patient?
ask for double confirmation that the patient was informed
ask if there was things ordered

if requires sign say pt got vaccine before but gave verbal consent to have the documentation correct

more invasive the procedure the more clear you need to be about the risks

Self-Directed Learning and Self-Attestations

Case 1 - Odd numbered groups

- You are getting ready to go out on rotations. The site sends a welcome email with items to be complete before starting the rotation (ex: get name badge, dress code, EHR training, etc.) You were quite busy around that time and didn't read it until past the due date. When you wanted to start your rotation, the site says you can't start until you are done, which will mean that you'll miss the first 2 days of the rotation.

Case 2 - Even numbered groups

- During the start of your rotation, you are to complete several online modules. Being as computer savvy as you are, you were able to find a way to complete the module without watching the module. 3 months later, you didn't put on proper protective gear in a contaminated situation. Upon investigation, it was discovered that you did not actually read the modules.

Self-Directed Learning and Self-Attestations

knowing what you can and cannot do, and dress code is important

Case 1 - Odd numbered groups

- How do you feel about the delay?
bad bc it was your fault - annoyed at self for missing and anxious about n important experiences and impression
- How do these tasks/requirements relate to patient care and patient safety?
need to know how things work so can integrate and not delay p and create extra errors esp if first time setting foot on site
- How can the student take responsibility and demonstrate growth moving forward?

own it - I'm so sorry I messed up and I won't do this in future

READ emails

let whoever is in charge know and own the mistake and will prove ourselves

Case 2 - Even numbered groups

- How do you feel?
unprofessionalism pt and provider safety
- What should happen to this student?
contextual, post-incidence review from all perspectives
- How can the student take responsibility and demonstrate growth moving forward?

recognize you possess ignorance and make sure to not repeat in the future hospitals have policies so could get kicked out if breach policy

Questions for both groups

- What does self-attestation mean?
- What does self-directed study mean?
- Why are these important professional values?

self-reporting good and bad missing some lectures to spend time with fam

stuff you are accountable for but won't necessarily be held to a specific timeline to complete

self-governing and need to hold to a high standard and make sure word is high quality and stays high quality

Questions?

Thank you!

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